Tamoxifen · daily hormone therapy

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Ask early for help with flushes, sweats, sleep, mood, vaginal or sexual symptoms, nausea or fatigue. A difficult side effect matters even if it is not medically severe. The team can discuss symptom treatment and the endocrine plan.
- This oral hormone treatment does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. Symptom medicines are chosen individually and checked for interactions.
- Agree who arranges the recommended eye and blood checks, their due windows and who reviews the results. Report bleeding or visual changes between appointments.
- The effect on bone differs before and after menopause. Discuss your own fracture risks and whether a bone-density test is appropriate; activity and bone care should suit your circumstances.
- Discuss the expected benefit in your treatment setting alongside clot, womb, eye and liver risks and effects on daily life. Tell the team about previous clots, severe liver disease and all other medicines. Tamoxifen is not contraception: discuss an effective non-hormonal method during treatment and for nine months after stopping, where relevant. Absent periods do not prove infertility. Pregnancy and breastfeeding are excluded from this NCCP pathway. Do not change antidepressants, blood thinners or other regular medicines yourself.
- NCCP: no expected renal/dialysis adjustment; no mild/moderate hepatic adjustment; severe hepatic impairment not recommended. Do not invent creatinine, aspartate aminotransferase (AST), bilirubin or generic toxicity dose ladders.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00253 v6 · Monitoring and treatment map